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Steven Posnack

Principal Deputy Assistant Secretary for Technology Policy
ASTP/ONC
Department of Health and Human Services
330 C Street SW
Washington, DC 20201

RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955–AA13)

Dear Mr. Posnack:

Overjet, Inc. appreciates the opportunity to respond to this Request for Information on accelerating the adoption and use of artificial intelligence (AI) in clinical care. Overjet is a dental artificial intelligence company headquartered in Boston, Massachusetts, that develops FDA-cleared AI solutions for the analysis of dental radiographic images. Our products are deployed across dental service organizations (DSOs), insurance carriers, individual dental practices, and government programs, where they assist clinicians in identifying dental pathology and supporting evidence-based treatment decisions. To date, Overjet's AI has been used to analyze tens of millions of dental radiographic images across thousands of practice locations nationwide.

This response draws on Overjet's direct experience as a developer, deployer, and continuous evaluator of clinical AI tools in one of the most widely practiced yet frequently overlooked domains of health care: dentistry. The perspectives offered here are informed by the expertise of our clinical and regulatory leadership, including prior federal service at the Centers for Medicare & Medicaid Services, the Food and Drug Administration, and the National Institutes of Health.

We commend HHS for framing this RFI around three actionable policy levers—regulation, reimbursement, and research & development—and for explicitly seeking experience-based input from AI developers and clinical users. Our comments address each of these domains and respond to specific questions posed in the RFI, with an emphasis on dental AI as a critical and underrepresented use case in national AI policy. Overjet is also submitting a separate response to the Diagnostic Imaging Interoperability RFI (RIN 0955–AA11), and we respectfully request that the two submissions be read as complementary.

Summary of Key Recommendations

Overjet respectfully urges HHS to consider the following priority actions as it develops policy to accelerate AI in clinical care:

- Create reimbursement pathways for AI-assisted dental diagnostics through new or add-on CPT/CDT codes and inclusion of AI-enabled quality metrics in value-based payment models, including Medicare Advantage Star Ratings.
- Support independent, university-based dental AI evaluation infrastructure through competitive research grants, benchmarking datasets, and cooperative agreements—moving beyond vendor-driven validation to build public trust and clinical evidence.
- Mandate inclusion of dental data classes in the United States Core Data for Interoperability (USCDI) and support interoperability standards for dental imaging to break down medical-dental data silos that limit AI's potential to improve systemic health outcomes.
- Prioritize federal research investment in medical-dental AI integration, AI-enabled quality measurement, health equity in dental AI, and longitudinal outcomes research.

I. Regulation

Question 1: Biggest Barriers to Private Sector Innovation in AI for Health Care

Overjet identifies three primary barriers to dental AI innovation and adoption:

Absence of dental-specific considerations in AI policy. Federal AI policy discussions—including FDA’s AI/ML-enabled device framework, ONC’s certification criteria, and CMS’s coverage determinations—have been developed primarily around hospital-based and physician-oriented use cases. Dentistry, which accounts for approximately 200,000 practicing clinicians in the United States, operates in a distinct clinical environment with different imaging modalities (intraoral periapical radiographs, bitewing radiographs, panoramic images, cone-beam computed tomography), different practice structures (predominantly small private practices and mid-size DSOs rather than large health systems), and different electronic health record ecosystems. AI policy that does not account for these differences risks creating frameworks that are poorly suited to dental care—or that inadvertently exclude dental AI from the benefits of federal support.

Lack of trusted benchmarks and evaluation infrastructure. There is currently no nationally recognized benchmarking framework for dental AI performance. Unlike radiology, where organizations such as the American College of Radiology have established AI validation processes, dental AI lacks equivalent infrastructure for independent evaluation, post-market surveillance, and comparative assessment. This gap creates uncertainty for purchasers and regulators alike and depresses adoption. HHS could catalyze progress by supporting the development of dental AI benchmarking standards, potentially through NIST collaboration or through competitive grants to academic dental institutions.

Question 2: Regulatory, Payment Policy, or Programmatic Design Changes HHS Should Prioritize

Overjet recommends four priority actions:

First, CMS should develop reimbursement pathways for AI-assisted dental diagnostics. Currently, there is no Current Procedural Terminology (CPT) code, Current Dental Terminology (CDT) code, or Healthcare Common Procedure Coding System (HCPCS) code that captures the distinct clinical value of AI-assisted radiographic analysis in dentistry. Overjet’s products assist clinicians in identifying caries, measuring bone levels, and supporting treatment planning—functions that improve diagnostic accuracy and reduce inter-examiner variability. These contributions have measurable value that is not captured by existing coding structures. We recommend that CMS work with the American Dental Association and AI developers to establish appropriate coding and coverage mechanisms. Options include a new CDT code for AI-assisted radiographic interpretation, an add-on code to existing diagnostic imaging codes (D0220, D0230, D0330), or inclusion of AI-assisted diagnostics in value-based dental payment models.

Second, HHS should ensure that Medicare Advantage organizations’ dental benefits are evaluated for quality and adequacy in a manner that accounts for AI-enabled quality

measurement. Recent analyses indicate that approximately 4.7 million Medicare Advantage beneficiaries have been affected by the erosion of supplemental dental benefits as plans have reduced coverage while continuing to market dental access as an enrollment incentive. This pattern highlights a structural gap: there is no quality measurement infrastructure in dental care that would allow CMS, beneficiaries, or plans themselves to objectively assess whether the dental care being delivered meets clinical standards. AI-assisted radiographic analysis can provide precisely this capability—objective, consistent measurement of diagnostic and treatment quality across large patient populations. We recommend that CMS explore the integration of AI-derived dental quality metrics into the Medicare Advantage Star Ratings framework or as a condition of supplemental benefit adequacy determinations. Without quality measurement, there is no mechanism to distinguish plans that deliver meaningful dental care from those that offer nominal benefits with limited clinical value.

Third, HHS should address the regulatory gap in dental AI by ensuring that FDA’s evolving Predetermined Change Control Plan framework, the total product lifecycle approach for AI/ML-enabled devices, and any forthcoming AI regulatory guidance explicitly include dental imaging AI within their scope. Dental AI products face the same continuous learning and adaptation challenges as other clinical AI tools, but the dental use case is rarely addressed in regulatory guidance documents. Explicit inclusion would provide dental AI developers with regulatory predictability and reduce the uncertainty that currently slows innovation. See 21 CFR Part 892 (Radiology) for relevant device classification frameworks.

Question 3: Novel Legal and Implementation Issues for Non-Medical Device AI

The dental AI market presents a particularly acute illustration of the governance challenges identified in this question. Dental practices are predominantly small businesses with limited legal, compliance, and IT infrastructure. When a dental practice acquires an AI tool—whether FDA-cleared or not—questions of clinical liability, data governance, and informed consent are often unaddressed by existing practice policies.

HHS can play a constructive role by issuing model guidance for dental practices on the clinical governance of AI tools, including recommended practices for informed consent, clinician override documentation, and adverse event reporting. We note that liability frameworks should distinguish between AI tools that have undergone FDA clearance (and thus have established intended uses, contraindications, and performance characteristics) and those that have not. This distinction is critical to establishing proportionate accountability structures.

Additionally, HIPAA’s current framework was not designed with AI-mediated image analysis in mind. When a dental image is transmitted to a cloud-based AI system for analysis, questions arise about the classification of AI-generated annotations (are they part of the patient’s designated record set?), the scope of business associate agreements, and the application of the minimum necessary standard to image data. We recommend three specific actions: (1) that the HHS Office for Civil Rights issue a FAQ specifically addressing AI-mediated clinical image analysis under HIPAA, including guidance on whether AI-generated annotations constitute protected health information within the designated record set; (2) that OCR develop model business associate agreement language for cloud-based clinical AI services; and (3) that HHS

clarify the applicability of the minimum necessary standard to image data transmitted for AI analysis, recognizing that clinical images inherently require full transmission for accurate interpretation.

II. Reimbursement

Question 5: Supporting Private Sector Accreditation, Certification, and Credentialing

HHS can most effectively support private sector AI governance activities by establishing a clear federal baseline against which voluntary programs can be measured. In the dental AI context, this means:

- Recognizing FDA clearance as the foundational tier of AI validation for products that analyze dental images, and communicating this standard clearly to payers, state regulators, and dental practices.
- Supporting the development of dental AI certification programs within existing accreditation bodies—such as the Commission on Dental Accreditation (CODA) or the Joint Commission—that go beyond FDA clearance to assess ongoing performance, clinical integration, and patient safety.
- Fostering models of independent, university-based clinical AI evaluation funded through competitive research grants. The emerging model of industry-funded, university-conducted competitive research—in which academic dental institutions apply for grants to independently evaluate and validate AI tools—offers a promising pathway for building clinical evidence that is credible, reproducible, and free from vendor bias. HHS could support this model through AHRQ, NIDCR, or HRSA mechanisms, or through cooperative research and development agreements (CRADAs) that pair AI developers with academic research centers.
- Encouraging dental schools to incorporate AI literacy into predoctoral and residency curricula, so that the next generation of dentists is equipped to evaluate, implement, and oversee AI tools in practice. HHS could support this through HRSA training grants or through updates to CODA accreditation standards.

Question 6: Where AI Tools Have Met or Exceeded Expectations

Overjet's deployments across dental service organizations and insurance carriers provide direct evidence of AI's capacity to improve clinical outcomes while reducing costs. Our AI has been used to analyze tens of millions of dental radiographic images, providing a substantial real-world evidence base:

Diagnostic consistency. In multi-provider dental organizations, AI-assisted radiographic analysis has measurably reduced inter-examiner variability in caries detection and bone loss measurement. Published literature demonstrates that inter-examiner agreement in dental radiographic interpretation is historically low, with kappa values often below 0.5 for caries detection. Overjet's AI provides a consistent, objective reference point that has been shown to narrow this variability, leading to more consistent treatment recommendations across clinicians and practice locations. In deployed settings, practices using Overjet's AI have reported meaningful improvements in diagnostic agreement rates across providers.

Utilization management. Dental insurance carriers using Overjet's AI for claims review have reported significant improvements in the accuracy and efficiency of radiographic review, including reduced reliance on manual clinical review for routine claims and the identification of billing patterns inconsistent with radiographic findings. These efficiencies directly reduce administrative costs while improving the clinical integrity of claims adjudication.

Quality measurement at scale. AI-assisted analysis enables the objective measurement of clinical quality across large patient populations—something previously impossible in dental care, where quality measurement has been limited to process measures and patient surveys rather than clinical outcome assessment. This capability represents a transformative advance: for the first time, payers and health systems can assess whether the diagnostic and treatment decisions made across their networks are consistent with radiographic evidence, enabling data-driven quality improvement at a population level.

Where AI has fallen short of expectations, the primary challenges have been related to integration complexity rather than analytical performance. Dental practices use a fragmented ecosystem of practice management systems, imaging software, and clinical workflows. The technical burden of integrating AI tools into these workflows remains significant, particularly for small practices. HHS could address this by supporting interoperability standards for dental imaging (as addressed in the companion Diagnostic Imaging Interoperability RFI, RIN 0955-AA11) and by incentivizing practice management software vendors to adopt open integration standards.

Question 7: Decision Makers and Administrative Hurdles to AI Adoption

In dental care, AI adoption decisions are influenced by a distinct set of stakeholders compared to hospital-based health care:

- In dental service organizations, clinical leadership (typically chief dental officers) and operations leadership jointly evaluate AI tools based on clinical value, workflow impact, and return on investment.
- In independent dental practices, the practicing dentist is typically the sole decision maker, and adoption decisions are heavily influenced by peer recommendations, dental society endorsements, and perceived ease of integration.
- In dental insurance, chief dental officers and medical directors evaluate AI tools for claims review and utilization management based on accuracy, regulatory compliance, and operational efficiency.

The primary administrative hurdles in dental AI adoption include the lack of reimbursement (clinicians cannot bill for AI-assisted analysis), the cost of integration with existing practice management systems, uncertainty about regulatory requirements, and the absence of professional society guidance on AI implementation. Each of these is amenable to federal policy intervention through the mechanisms described throughout this response.

III. Research & Development

Question 4: Promising AI Evaluation Methods

Overjet's experience across tens of millions of dental image analyses has demonstrated the importance of several evaluation approaches:

Pre-deployment clinical validation against expert consensus panels. Our FDA clearance process involved validation studies comparing AI performance to panels of licensed dental specialists. This approach—using independent, multi-examiner expert consensus as the reference standard rather than single-examiner ground truth—is critical in dental imaging where inter-examiner variability is well-documented.

Post-deployment performance monitoring with demographic stratification. We continuously monitor our AI's performance across patient populations, practice types, and imaging equipment to identify and address performance variations. This is particularly important in dental care, where imaging quality varies significantly between practices and between digital sensor manufacturers.

Workflow integration assessment. Technical accuracy alone is insufficient. We evaluate how AI outputs are used by clinicians in practice—including rates of clinician agreement, override patterns, and impacts on treatment planning. These human-centered metrics are essential for understanding real-world clinical value.

HHS should support the development of these evaluation methods through competitive grants to academic dental institutions, cooperative agreements with industry, and the creation of dental AI-specific evaluation datasets. Prize competitions, as mentioned in the RFI, could be particularly effective for encouraging innovation in dental AI validation methodologies.

Question 8: Where Enhanced Interoperability Would Accelerate AI Development

Enhanced interoperability would profoundly accelerate dental AI development and deployment. Three specific areas merit priority attention:

Dental imaging standards. Dental radiographic images are frequently stored in proprietary formats within practice management systems, with no standardized exchange pathway. While medical imaging benefits from mature DICOM infrastructure, dental imaging often lacks consistent DICOM implementation. The adoption of standardized image formats and exchange protocols for dental imaging would expand the data available for AI development and enable interoperable deployment across practice management systems. We address this in greater detail in our response to the Diagnostic Imaging Interoperability RFI (RIN 0955-AA11).

Clinical data integration across medical and dental records. Dental findings have significant implications for systemic health—including the detection of carotid artery calcifications on panoramic radiographs, oral manifestations of diabetes and autoimmune conditions, and medication-related oral health risks such as xerostomia. AI tools that bridge medical and dental data require interoperable data standards that currently do not exist. The inclusion of dental

data classes in the United States Core Data for Interoperability (USCDI) would be a critical step. Preliminary analyses suggest that systematic AI-enabled screening of existing dental panoramic radiographs for incidental findings such as carotid artery calcifications could identify individuals at elevated cardiovascular risk at a fraction of the cost of dedicated screening programs—representing a significant opportunity for early intervention and cost savings that is currently lost due to the medical-dental data divide.

Benchmarking datasets. The development of curated, de-identified dental imaging datasets with expert-validated annotations would enable independent benchmarking of dental AI products, comparative effectiveness research, and the training of next-generation AI models. HHS could support this through NIH or NIDCR funding mechanisms.

Question 9: Patient and Caregiver Perspectives on AI

Patients and caregivers stand to benefit substantially from dental AI in several ways. Chief among these is the problem of diagnostic variability: it is well-documented that patients can receive significantly different diagnoses and treatment recommendations depending on which dentist they visit, a phenomenon that has been the subject of investigative journalism and academic research alike. AI-assisted analysis provides an objective, evidence-based reference point that can reduce unwarranted treatment variation and give patients greater confidence that recommended treatments are consistent with radiographic findings.

Additional patient benefits include visual explanations of dental conditions that improve health literacy and informed consent, quality assurance that treatment recommendations are evidence-based, and the potential for AI-enabled second opinions that are accessible and affordable. These benefits are particularly important for vulnerable populations—including elderly patients, individuals with limited English proficiency, and those in underserved communities—who may have fewer resources to evaluate whether recommended treatments are appropriate.

Patient concerns about dental AI center on data privacy (particularly for images of their face and teeth), the fear that AI will replace clinical judgment rather than augment it, and the lack of transparency about when and how AI is being used in their care. HHS can address these concerns by encouraging the development of patient-facing AI disclosure standards, supporting patient education initiatives, and ensuring that AI tools are deployed as clinical decision support rather than autonomous diagnostic systems.

Question 10: Priority Areas for AI Research

Overjet recommends that HHS prioritize AI research in the following areas:

- **Medical-dental AI integration:** Research on AI systems that bridge dental imaging findings with systemic health conditions, including the use of dental panoramic radiographs for screening of carotid artery calcifications, osteoporosis, and airway abnormalities. The return on investment for these applications is potentially substantial: dental panoramic radiographs are already acquired as part of routine dental care for millions of Americans annually, and AI-enabled analysis of these existing images for

incidental systemic findings could enable population-level screening at minimal marginal cost.

- **AI-enabled quality measurement in dental care:** Development and validation of AI-derived quality measures for dental care that can be used in value-based payment programs, accreditation, and public reporting. This research is foundational to modernizing dental payment systems and ensuring accountability for clinical quality.
- **Health equity in dental AI:** Research on the performance and impact of dental AI across diverse patient populations, including evaluation of potential algorithmic bias related to demographic factors, imaging equipment variations, and practice settings.
- **Longitudinal outcomes research:** Studies examining how AI-assisted dental diagnostics affect patient outcomes over time, including the impact on disease progression, treatment appropriateness, and health care costs.
- **Medication-related oral health impacts:** Research on AI-driven risk assessment tools that analyze patient medication profiles to identify individuals at elevated risk for oral health complications such as xerostomia, which affects millions of Americans taking common medications and significantly increases the risk of caries, periodontal disease, and oral infections.

Published findings on dental AI indicate that these tools can achieve performance comparable to or exceeding specialist-level diagnosis for certain conditions, including caries detection and periodontal bone loss measurement. However, the literature on real-world clinical impact—as distinct from analytical performance—remains limited. HHS should prioritize implementation science research that evaluates dental AI in practice settings rather than laboratory conditions alone.

Conclusion

Overjet commends HHS for undertaking this comprehensive inquiry into accelerating AI in clinical care. We urge the Department to ensure that dental AI is explicitly included in the regulatory frameworks, reimbursement pathways, and research agendas that emerge from this process. Dentistry serves the vast majority of Americans, involves complex diagnostic imaging, and stands to benefit enormously from responsible AI deployment—yet it has been largely absent from federal AI policy discussions to date.

The dental AI use case is not peripheral to the goals of this RFI—it is central. Dental care is one of the few clinical domains where AI has already been deployed at scale, subjected to FDA review, integrated into routine clinical workflows, and demonstrated measurable impact on diagnostic quality and cost efficiency. The lessons learned from dental AI deployment can inform broader federal AI policy, and the barriers facing dental AI are illustrative of the challenges that will confront clinical AI adoption across all specialties.

Overjet is prepared to serve as a resource to HHS as it develops policy in this domain. We bring the perspective of a company that has navigated FDA clearance, deployed AI across multiple market channels, and continuously evaluated real-world performance—and we are led by individuals with deep experience in federal health policy, dental clinical practice, and AI development.

We welcome the opportunity to discuss these comments further and to provide additional evidence or technical detail as requested.

Respectfully submitted,

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